Royal Devon University Healthcare NHS Foundation Trust

Medical Product Evaluation and Introduction Policy

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Please *specify* standard/criterion numbers and tick ✓ other boxes as appropriate

Monitoring Information		**Strategic Directions – Key Milestones**	
Patient Experience	✓	Maintain Operational Service Delivery	
Assurance Framework		Integrated Community Pathways	
Monitor / Finance / Performance	✓	Develop Acute services	
CQC Fundamental Standards - Regulation**: Outcome 11**		Infection Control	
Other *(please specify)*:	Research and Development, Healthcare Standards (CORE)		
Note: This document has been assessed for any equality, diversity or human rights implications			

Controlled document

This document has been created following the Royal Devon University Healthcare NHS Foundation Trust's Policy for Procedural Documents. It should not be altered in any way without the express permission of the author or their representative.

Full History		Status: Final	
Version	Date	Author	Reason
1.0	01/2010	Supplier Development Advisor	New Policy.
2.0	15/01/2014	Supplier Development Advisor	Review of content, and policy reformatted to comply with the new Policy template.
3.0	18/04/2018	Head of Procurement & Commercial Support	Review of content and template to comply with Trust formats. Economic analysis (document savings) amended to savings forum to simplify and reduce duplication of work. The flowchart (appendix 2) changed to ensure historical information within the Trust has been captured correctly.
4.0	18/09/2025	Deputy Head of Procurement	Reviewed, put into new format and updated to reflect changes to processes without the Medical Equipment and products Group (MEPG) and referencing radiation protection in sections 1.3 and 5.2.

Associated Trust Policies / Procedural documents:	Decontamination Policy and Procedures Point of care testing, procurement and ongoing quality assurance policy Medical Equipment and Devices Management and Training Policy Research and Development Policy
Key Words:	Medical, product, evaluation, introduction, MEPG, MDSG, devices, trial, implementation
In consultation with and date: Procurement Management Team: 08/08/2025 Research and Development: 21/08/2025 Infection Control: 21/08/2025 Medical Equipment Manager: 21/08/2025 HSDU: 21/08/2025 Patient Equality Lead: 22/08/2025 Trust Medical Director: 22/08/2025 Medical Physics: 18/09/2025 Medical Devices Steering Group: 18/09/2025	
Contact for Review:	Deputy Head of Procurement
Executive Lead Signature: *(Applicable only to Trust Strategies & Policies)*	

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KEY POINTS OF THIS POLICY:

This policy documents the decision-making process for the introduction of new medical equipment and products, ensuring they are safe for patients and staff, are clinically effective, cost effective, and where possible, are standardised throughout the Royal Devon University Healthcare NHS Foundation Trust (hereafter referred to as “the Trust”).

1 INTRODUCTION

1.1 The decision to carry out a formal evaluation of a product at the Trust will depend on the criticality of the product, the area/areas where it is to be used, the risk of failure, the cost implications, and the national directive on the product or equipment. Not all implementations may require a formal evaluation.

1.2 Medical products and devices to be introduced should be approved by a suitable clinician.

1.3 The key principle is that the appropriate people should be involved in the selection and adoption of clinical products. These people may include, and is not limited to;

- Clinical Departments
- Clinical Nurse Specialist Teams (Appendix 1)
- Clinical Engineering (MEM / EBME)
- Learning & Development (Clinical Skills)
- Governance
- Research & Development
- Hospital Sterilisation and Decontamination Unit (HSDU)
- Estates
- Procurement
- Medical Physics (Radiation Protection Advisers and Medical Physics Experts)

1.3.1 In Scope;

- **Replacement of products as part of a Procurement process** – The engagement of the department or specialist group is part of the normal Procurement process. This is a pre-planned tendering decision among Procurement, the clinical decision makers and others.
- **New Product Requests**.
- **Research and Development activity**.
- **Other adoptions or products not controlled via formal Procurement processes**.

1.3.2 Out of scope;

- **Replacement suitably similar product or range as a result of unavailability** – Time is usually short; procurement and logistics will work with the clinical department to identify and procure suitable alternatives at short notice. The Procurement and the user cohort will decide who will be responsible for approving the alternative. This may be the care group directors or a clinical specialist or a specific department.
- **Medicines** – as defined by the Medicines and Healthcare Products Regulatory Agency (MHRA).
- **Non-clinical products**.

1.4 This document supports work carried out by the Trust including the Research and Development Department (R&D) as well as the Procurement department.

1.5 The Trust is required to demonstrate compliance with:

- Medicines and Healthcare Products Regulatory Agency (MHRA)
- Clinical Negligence Scheme for Trusts (CNST)
- Trust Decontamination Policy

2 PURPOSE

2.1 The purpose of this policy is to document the decision-making process for the introduction of new medical equipment and products, ensuring they are safe for patients and staff, are clinically effective, cost effective, and where possible, are standardised throughout the Trust.

3 DEFINITIONS

3.1 **Medical Device** – an instrument, apparatus, appliance, material or other article, whether used alone or in combination, together with any software necessary for its proper application, which —

(a) is intended by the manufacturer to be used for human beings for the purpose of-
- (i) diagnosis, prevention, monitoring, treatment or alleviation of disease,
- (ii) diagnosis, monitoring, treatment, alleviation of or compensation for an injury or handicap,
- (iii) investigation, replacement or modification of the anatomy or of a physiological process, or
- (iv) control of conception; and

(b) does not achieve its principal intended action in or on the human body by pharmacological, immunological or metabolic means, even if it is assisted in its function by such means,

and includes devices intended to administer a medicinal product or which incorporate as an integral part a substance which, if used separately, would be a medicinal product and which is liable to act upon the body with action ancillary to that of the device (*The Medical Devices Regulations 2002*).

3.2 **Medicines** – Article 1 of Directive 2001/83/EC as amended defines a "medicinal product" as:

"Any substance or combination of substances presented as having properties for treating or preventing disease in human beings; [the first / presentational limb].

Any substance or combination of substances which may be used in, or administered to, human beings, either with a view to restoring, correcting or modifying physiological functions by exerting a pharmacological, immunological or metabolic action, or to making a medical diagnosis" [the second / functional limb].

4 DUTIES AND RESPONSIBILITIES OF STAFF

4.1 **As Executive Lead, the Trust Medical Director** has overall responsibility for the implementation of this policy.

4.2 **Clinical departments** are responsible for the safe and effective implementation and use of products used within their areas.

4.3 The **Procurement Lead** is responsible for working with the Evaluation Lead and helping to develop the any Procurement and evaluation plans for products being considered. The Procurement Lead will complete the Trust Cost change form, logging the evaluation outcome.

4.4 The **Evaluation Lead** is proposing a product change is responsible for ensuring that all relevant stakeholders are informed. They are responsible for coordinating Clinical staff throughout the evaluation. All stakeholders agree to work on the evaluation.

5 REQUESTING A PRODUCT CHANGE

5.1 The Proposer will be required to contact their Care Group Procurement team for their area and provide the following details:

- New product details; product code, manufacturer.
- Research, if any, pertaining to the effectiveness and cost-effectiveness of the product (this could include published reports, reviews, research papers and/or the proposer's own work).
- Argument for the use of the product.
- Details of product to be replaced, if relevant; product code, manufacturer.
- Cost considerations including consumables and maintenance.
- Other sites where the product is being used.

5.2 The following should be considered:

- Patient care
- Maintenance/testing
- Identify similar devices already in use – range of equipment should be kept to a minimum to help maintain standardisation across Devon
- Supply routes and ordering processes
- Training required
- Manual handling
- Waste management
- Life expectancy of the equipment
- UKCA marking or the CE marking
- Areas the device is to be used – Consider departments, wards or patient groups.
- Decontamination processes required
- Associated costs (not a complete list);
    - Old stock to be written off
    - Disposal of old equipment
    - Cleaning
    - Other
- Risks – listed and scored
- Radiation Protection (if required)

5.3 In addition, the Procurement Lead will identify if:

- Indemnity is required (for suppliers providing loaned or free of charge equipment to NHS organisations in England).
- A committed contract already exists.
- Work is already underway.

5.4 With all relevant stakeholders are involved, a single decision should be made on behalf of all departments in the Trust effected by the proposed change to ensure standardisation, clinical best practice and cost efficiency is maintained.

6 PRODUCT EVALUATION

6.1 The evaluation will capture the essential requirements to ensure the product being assessed is clinically suitable and good value. The decision to carry out a formal evaluation will depend on the criticality of the product, the area / areas where it is to be used, the risk of failure, the cost implications, and the national directive on the product or equipment. Not all implementations may require a formal evaluation.

7 ARCHIVING ARRANGEMENTS

7.1 The original copy of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet until it is superseded. Archived electronic copies will be stored on the Trust's 'TRUST WIDE POLICY ADMIN' shared drive and will be held indefinitely

8 PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY

8.1 To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Evaluation of medical devices undertaken by the Trust.	Evaluation documents to be held in Procurement.	Chair of Medical Devices Steering Group (MDSG).
Evaluation of products organised as part of a Trust tender process.	Evaluation outcome documented as part of tender.	Chair of Medical Devices Steering Group (MDSG).

9 REFERENCES

Medicines & Healthcare products Regulatory Agency (MHRA) (2003). London: DoH. Available at: https://www.gov.uk/government/organisations/medicines-and-healthcare-products-regulatory-agency

The Medical Device Regulations 2002 (SI 2002/618). London: Stationery Office. Available at: http://www.legislation.gov.uk/uksi/2002/618/made

Department of Health (2010) *Master Indemnity Agreement (MIA)* (Guidance). London: DoH. Available at: http://www.dh.gov.uk/en/Publicationsandstatistics/Publications/PublicationsPolicyAndGuidan

ce/DH_117175

Clinical Negligence Scheme for Trusts - NHS Resolution (CNST). London: DoH. Available at: https://resolution.nhs.uk/services/claims-management/clinical-schemes/clinical-negligence-scheme-for-trusts/

10 APPENDIX 1: CLINICAL NURSE SPECIALIST TEAM PRODUCT CATEGORIES

Nurse Specialist Team	Product Categories	Key CNS Contact
Tissue Viability Team (TV)	• Wound care dressings • Pressure relieving devices (Repose products and Heel boots etc.) • Negative Pressure Wound Therapy • Anti-Embolism Stockings	rduh.tissueviability-eastern@nhs.net rduh.tissueviability-northern@nhs.net
Infections Control (IPC)	• PPE (non-covid) • Exam Gloves • Wipes (wet, dry and cleaning) • Skin Disinfectant and Equipment/environment cleaning products • Clinical Waste Containers (sharps bins) • Pulp products • Hand hygiene (Gel, soap and moisturiser/hand towels)	rduh.infectioncontrol-northern@nhs.net rduh.infectioncontrol-eastern@nhs.net
Nutrition and Dietetics	Nutrition • Enteral Feeding devices (NG/Ryles Tubes, Enteral Syringes, Gastro Buttons etc.) Dietitians • Oral Nutritional supplements (ONS) • All foods supplied to clinical areas	rduh.dietetics-northern@nhs.net catherine.tancock@nhs.net
Vascular Access Team (VAT)	• Needle-free devices • Cannulas • PICC / Midlines • IV Film Dressings • Tourniquets	rduh.vatexeter@nhs.net
Clinical Measurements Team	• Urology Products i.e. Catheters (excluding what is used in Theatres) • Urine Drainage Bags / Leg bags • All continence related products	susan.cole4@nhs.net
Resuscitation Lead	• Non-rebreathe Oxygen Masks • Oropharyngeal Airways • Bag Valve Masks • Defibrillator Pads	rduh.resuscitation@nhs.net rduh.resusofficers@nhs.net
Pain Management Team	• PCA giving sets	Inpatient: ndht.inpatientpain@nhs.net Outpatient: ndht.pain@nhs.net

11 APPENDIX 2: COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the strategy/policy	All staff that purchase goods or services for the Trust as well as all staff that come in contact with external suppliers and commercial organisations.
The key changes if a revised policy/strategy	Reviewed, put into new format and updated to reflect changes to processes without the Medical Equipment and products Group (MEPG).
The key objectives	The purpose of this policy is to document the decision-making process for the introduction of new medical equipment and products, ensuring they are safe for patients and staff, are clinically effective, cost effective and where possible, are standardised throughout the Trust.
How new staff will be made aware of the policy and manager action	Through required cascade of information from MDSG, highlight policy at team meetings, advertise on Hub.
Specific Issues to be raised with staff	Encouragement to be a proposer of any product changes to encourage innovation.
Training available to staff	Support in this process available from the Procurement department.
Any other requirements	N/A
Issues following Equality Impact Assessment (if any)	No negative impacts. Positive impact of simplification of process.
Location of hard / electronic copy of the document etc.	The original copy of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet until it is superseded. Archived electronic copies will be stored on the Trust's 'TRUST WIDE POLICY ADMIN' shared drive and will be held indefinitely.

12 APPENDIX 3: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	Medical Product Evaluation and Introduction Policy
Division/Directorate and service area	Trust Wide
Name, job title and contact details of person completing the assessment	Nick Hodgson, Deputy Head of Procurement
Date completed:	21 August 2025

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

The purpose of this policy is to document the decision-making process for the introduction of new medical equipment and products, ensuring they are safe for patients and staff, are clinically effective, cost effective, and where possible, are standardised throughout the Trust.

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☐ Staff ☒ Patients ☐ Other (please specify)

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	☐	☒
Disability	☐	☒
Sex - *including: Transgender, and Pregnancy / Maternity*	☐	☒
Race	☐	☒
Religion / belief	☐	☒
Sexual orientation – *including: Marriage / Civil Partnership*	☐	☒

4. **Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to…** (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

Will not adversely affect any groups in society.

5. **Do you think the document meets our human rights obligations?** ☒
Feel free to expand on any human rights considerations in question 6 below.

A quick guide to human rights:
• ***Fairness –*** *how have you made sure it treat everyone justly?* • ***Respect –*** *how have you made sure it respects everyone as a person?* • ***Equality –*** *how does it give everyone an equal chance to get whatever it is offering?* • ***Dignity –*** *have you made sure it treats everyone with dignity?* • ***Autonomy –*** *Does it enable people to make decisions for themselves?*

6. **Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?**

Consultation widely with all groups. *Consulted with the Procurement Management Team, Medical Devices Steering Group and individual departments as listed on page 2 of this policy. Also consulted with the Inclusion Team and Sharifa Hashem and Sajjad Iqbal. The policy does not adversely affect any groups in society.*

7. **If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact** please note this below and how this will be monitored / addressed.

"Protected characteristic":	
Issue:	
How is this going to be monitored/ addressed in the future:	
Group that will be responsible for ensuring this carried out:	